OCCUPATIONAL ENGLISH TEST

OFFICIAL OET PREPARATION MATERIALS



Practice Book

Nursing

Set 1

SPEAKING

OCCUPATIONAL ENGLISH TEST

ISBN: 978-0-9807653-8-0 © Copyright Cambridge Boxhill Language Assessment.

This work is copyright. Apart from any use permitted under the Copyright Act 1968, no part may be reproduced by any process without prior written permission from Cambridge Boxhill Language Assessment.

For information in regards to OET visit the OET website: **www.occupationalenglishtest.org**

First published March 2010

2nd editon May 2015

Contents

An overview of OET (Occupational English Test)		1
History of the test		3
Description of OET		4
How the test is scored		6
Section 1:	What is the Speaking sub-test?	7
Section 2:	Assessment criteria	9
Section 3:	Information about role-play cards	23
Section 4:	Practice Test 1 (Role-play 1 and 2)	27
Section 5:	Practice Test 2 (Role-play 1 and 2)	31
Section 6:	Practice Test 3 (Role-play 1 and 2)	35
Section 7:	Further practice	38

An overview of OET (Occupational English Test)

OET assesses the language proficiency of healthcare professionals who wish to register and practise in an English-speaking environment. It is designed to meet the specific English language needs of the healthcare sector.

OET tests candidates from the following 12 health professions: Dentistry, Dietetics, Medicine, Nursing, Occupational Therapy, Optometry, Pharmacy, Physiotherapy, Podiatry, Radiography, Speech Pathology and Veterinary Science.

The test is now owned by Cambridge Boxhill Language Assessment Pty Ltd (CBLA), a venture between Cambridge English Language Assessment and Box Hill Institute in Australia.

Who recognises OET?

OET results are accepted as proof of English proficiency by healthcare boards and councils in Australia, New Zealand, Singapore, Namibia and Dubai for registration; by leading healthcare employers and educators for admission and recruitment; and Australian and New Zealand immigration.

For more information visit: www.occupationalenglishtest.org

When and where is OET available?

OET is available up to twelve times a year, in 35 countries.

What is in the test?

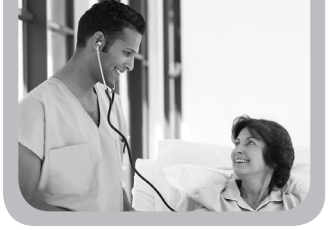
OET is an in-depth and thorough assessment of all areas of language ability – with an emphasis on communication in medical and health professional settings.

The test consists of four sub-tests:

- Listening
- Reading
- Writing
- Speaking

The Writing and Speaking sub-tests are specific to each profession, while the Listening and Reading sub-tests are common to all professions.

Sub-test (duration)	Content	Shows candidates can:
Listening (50 minutes)	2 tasks Common to all 12 professions	follow and understand a range of health-related spoken materials such as patient consultations and lectures.
Reading (60 minutes)	2 tasks Common to all 12 professions	read and understand different types of text on health-related subjects.
Writing (45 minutes)	1 task Specific to each profession	write a letter in a clear and accurate way which is relevant for the reader.
Speaking (20 minutes)	2 tasks Specific to each profession	effectively communicate in a real-life context through the use of role plays.



Results

OET Statements of Results include a separate grade for each sub-test, ranging from A (highest) to E (lowest). There is no overall grade for OET.

High-quality, secure language assessment

CBLA is committed to the highest standards of quality, security and integrity for OET – from test development, test delivery and results processing, through to post-examination review and evaluation.

Fair and consistent delivery of OET is ensured by secure technology and the continual training and monitoring of assessors, as well as test centre management and facilities.

The Writing and Speaking sub-tests are developed in consultation with practising healthcare professionals and educators to ensure test materials simulate real-life clinical situations, such as explaining a diagnosis and writing referral letters.

The Listening and Reading sub-tests are developed by the Language Testing Research Centre (LTRC) at the University of Melbourne.

Assessment

All sub-tests are assessed at the OET Centre by trained expert assessors. Assessment procedures routinely include double marking and statistical analysis to ensure that candidate results are accurate and fair.

Each of the four sub-tests is assessed in a specific way. Read more about OET assessment procedures at: www.occupationalenglishtest.org

Registration procedures

For registration details visit: www.occupationalenglishtest.org

Here you'll find all the information and instructions you need to apply for OET online for the first time, including test fees, ID, payment and photo guidelines.

Our ID procedures have DIBP approval, ensuring confidence in candidate identity.

Special provision

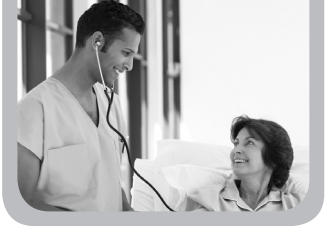
Candidates with special needs may apply in advance for special provision. CBLA makes all reasonable arrangements to accommodate special visual or auditory needs, including enlargement of print texts and special auditory equipment.

Preparation materials

Resources can be accessed from the OET website, including:

- sample papers
- suggested reading material
- a list of preparation training providers*

* This list is for information only – we do not endorse any particular training program.



History of the test

Occupational English Test was designed by Professor Tim McNamara of the University of Melbourne under contract to the Australian Federal Government.

As part of the annual intake of refugees and immigrants, hundreds of overseas-trained health practitioners were entering Australia by the mid to late 1980s. The majority were medical practitioners, but a number of other health professional groups were also represented.

The process of registration to practise in most health professions in Australia included three stages of assessment: English language proficiency, a multiple choice test of profession-specific clinical knowledge and a performance-based test of clinical competence.

Dissatisfaction with the results of existing language tests led to the development of thoroughly researched specifications for a communicative, contextualised test. OET has been frequently reviewed and analysed in the literature since the 1980s. McNamara (1996)[1] gives a full account of the development of the test and associated validation research.

The initial development of the test specifications involved:

a. extensive consultation with expert informants, including clinical educators, ESL teachers offering language support in clinical settings, and overseas-trained professionals who were completing or had completed a clinical bridging program.

b. literature search.

c. direct observation of the workplace.

Stages of Test Development (presentation by Prof Tim McNamara, August 2007)[2]

STAGES OF TEST DEVELOPMENT

- *Analysing target domain via 'job analysis'*

Job analysis (1): Consultation with expert informants

- **Clinical educators** (especially those with experience of overseas-trained health professionals) [via interview]
- **ESL teachers** offering language support in clinical settings [via interview]
- **Overseas-trained health professionals** who are in or have completed a clinical bridging programme [via questionnaire]

Job analysis (2 and 3):

- *Literature search*
- *Direct observation of the workplace*

Commonalities observed: (1) stages (some optional) of the 'consultation'

- 1. Assessment of the patient ('subjective assessment') including history taking.
- 2. Physical examination.
- 3. Explanation to patient of diagnosis and prognosis and course of treatment.
- 4. Treatment.
- 5. Patient/client/relative education and counselling.

This event provided the basis for specifications for a common format for assessment of *speaking* and *listening* skills.

Commonalities observed (2)

- Listening:
  - professional development
- Reading:
  - Professional development
  - Case history notes
- Writing:
  - Letters of referral

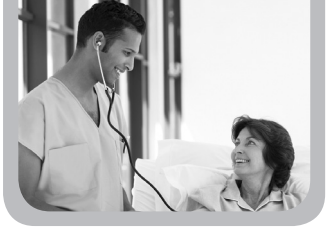
STAGES OF TEST DEVELOPMENT

- *Identifying resources/constraints:*

  Needed to maximize both
  - ***Specificity**: plausibility in professional context*

  and
  - ***Commonality**: limits to affordability of profession-specific materials*
- *Drafting of test specifications*

1. McNamara, T. [1996] Measuring Second Language Performance. London: Longman.
2. McNamara, T. [2007] Stages of Test Development. OET Forum.



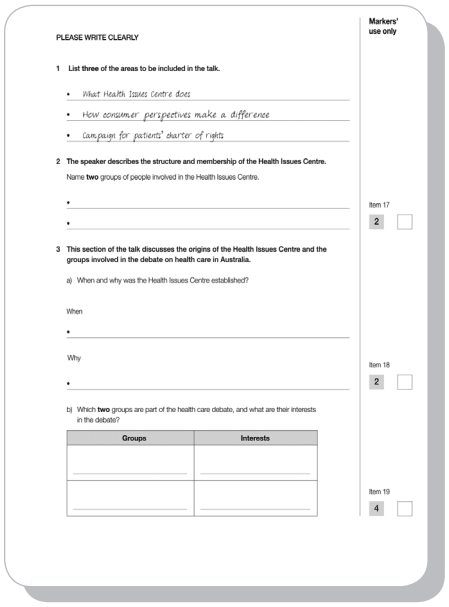
Description of OET

Test format

OET assesses listening, reading, writing and speaking.

There is a separate sub-test for each skill area. The Listening and Reading sub-tests are designed to assess the ability to understand spoken and written English in contexts related to general health and medicine. The sub-tests for Listening and Reading are common to all professions.

The Writing and Speaking sub-tests are specific to each profession and are designed to assess the ability to use English appropriately in a relevant professional context.



PLEASE WRITE CLEARLY

Markers' use only

1 **List three** of the areas to be included in the talk.

- What Health Issues Centre does
- How consumer perspectives make a difference
- Campaign for patients' charter of rights

2 **The speaker describes the structure and membership of the Health Issues Centre.**

Name **two** groups of people involved in the Health Issues Centre.

•

•

Item 17 2

3 **This section of the talk discusses the origins of the Health Issues Centre and the groups involved in the debate on health care in Australia.**

a) When and why was the Health Issues Centre established?

When

•

Why

•

Item 18 2

b) Which **two** groups are part of the health care debate, and what are their interests in the debate?

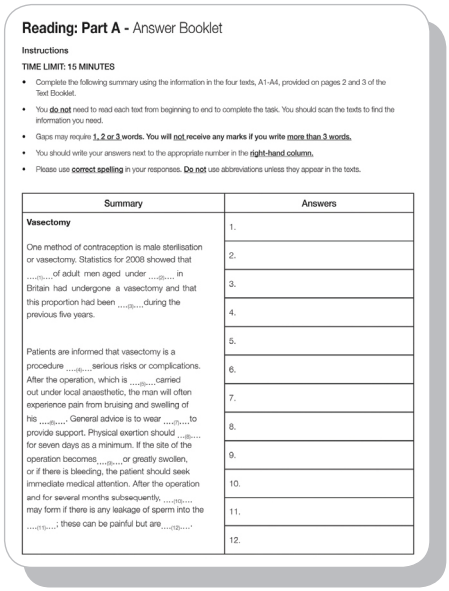
Groups	Interests

Item 19 4

Listening sub-test

The Listening sub-test consists of two parts: a recorded, simulated professional-patient consultation with note-taking questions (Part A), and a recorded talk or lecture on a health-related topic with short-answer/note-taking questions (Part B), each about 15 minutes of recorded speech. A set of questions is attached to each section and candidates write their answers while listening. The original recording is edited with pauses to allow candidates time to write their answers.

The format for Part A (the consultation) requires candidates to produce case notes under relevant headings and to write as much relevant information as possible. Part B (the lecture) requires candidates to complete a range of open-ended and fixed-choice listening tasks.



Reading: Part A - Answer Booklet

Instructions

TIME LIMIT: 15 MINUTES

- Complete the following summary using the information in the four texts, A1-A4, provided on pages 2 and 3 of the Text Booklet.
- You **do not** need to read each text from beginning to end to complete the task. You should scan the texts to find the information you need.
- Gaps may require **1, 2 or 3 words. You will not receive any marks if you write more than 3 words.**
- You should write your answers next to the appropriate number in the **right-hand column.**
- Please use **correct spelling** in your responses. **Do not** use abbreviations unless they appear in the texts.

Summary	Answers
Vasectomy One method of contraception is male sterilisation or vasectomy. Statistics for 2008 showed that ….(1)….of adult men aged under ….(2)…. in Britain had undergone a vasectomy and that this proportion had been ….(3)….during the previous five years. Patients are informed that vasectomy is a procedure ….(4)…. serious risks or complications. After the operation, which is ….(5)…. carried out under local anaesthetic, the man will often experience pain from bruising and swelling of his ….(6)….. General advice is to wear ….(7)….to provide support. Physical exertion should ….(8)…. for seven days as a minimum. If the site of the operation becomes….(9)…. or greatly swollen, or if there is bleeding, the patient should seek immediate medical attention. After the operation and for several months subsequently, ….(10)…. may form if there is any leakage of sperm into the ….(11)…., these can be painful but are….(12)….	1. 2. 3. 4. 5. 6. 7. 8. 9. 10. 11. 12.

Reading sub-test

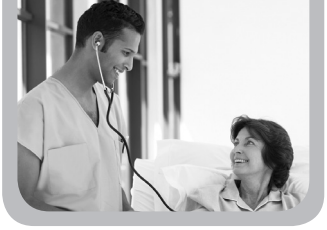
The Reading sub-test consists of two parts:

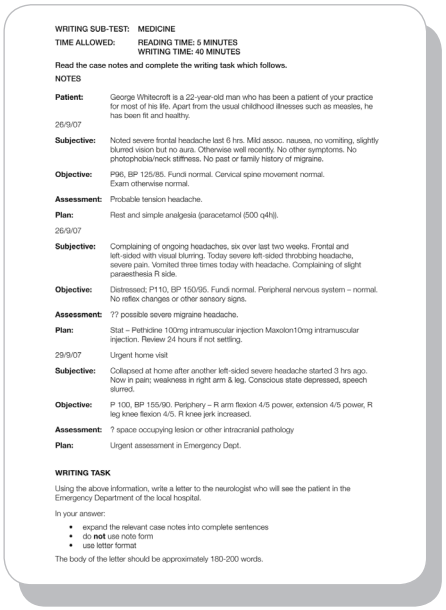
Part A is a summary reading task. This requires candidates to skim and scan 3-4 short texts (a total of about 650 words) related to a single topic and to complete a summary paragraph by filling in the missing words. Candidates are required to write responses for 25-35 gaps in total, within a strictly monitored time limit of 15 minutes.

Part A is designed to test the reader's ability to source information from multiple texts, to synthesise information in a meaningful way and to assess skimming and scanning ability within a time limit.

In Part B candidates are required to read two passages (600-800 words each) on general medical topics and answer 8-10 multiple-choice questions for each text (a total of 16-20 questions) – within a time limit of 45 minutes.

Part B is designed to test the reader's ability to read in greater detail general and specific information for comprehension.





WRITING SUB-TEST: MEDICINE
TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

NOTES

Patient: George Whitecroft is a 22-year-old man who has been a patient of your practice for most of his life. Apart from the usual childhood illnesses such as measles, he has been fit and healthy.

26/9/07

Subjective: Noted severe frontal headache last 6 hrs. Mild assoc. nausea, no vomiting, slightly blurred vision but no aura. Otherwise well recently. No other symptoms. No photophobia/neck stiffness. No past or family history of migraine.

Objective: P96, BP 125/85. Fundi normal. Cervical spine movement normal. Exam otherwise normal.

Assessment: Probable tension headache.

Plan: Rest and simple analgesia (paracetamol (500 q4h)).

26/9/07

Subjective: Complaining of ongoing headaches, six over last two weeks. Frontal and left-sided with visual blurring. Today severe left-sided throbbing headache, severe pain. Vomited three times today with headache. Complaining of slight paraesthesia R side.

Objective: Distressed; P110, BP 150/95. Fundi normal. Peripheral nervous system – normal. No reflex changes or other sensory signs.

Assessment: ?? possible severe migraine headache.

Plan: Stat – Pethidine 100mg intramuscular injection Maxolon10mg intramuscular injection. Review 24 hours if not settling.

29/9/07 Urgent home visit

Subjective: Collapsed at home after another left-sided severe headache started 3 hrs ago. Now in pain; weakness in right arm & leg. Conscious state depressed, speech slurred.

Objective: P 100, BP 155/90. Periphery – R arm flexion 4/5 power, extension 4/5 power, R leg knee flexion 4/5. R knee jerk increased.

Assessment: ? space occupying lesion or other intracranial pathology

Plan: Urgent assessment in Emergency Dept.

WRITING TASK

Using the above information, write a letter to the neurologist who will see the patient in the Emergency Department of the local hospital.

In your answer:

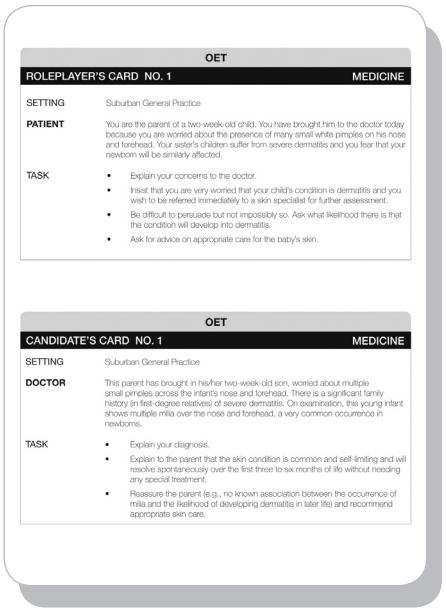
- expand the relevant case notes into complete sentences
- do **not** use note form
- use letter format

The body of the letter should be approximately 180-200 words.

Writing sub-test

The Writing sub-test usually consists of a scenario presented to the candidate, which requires the production of a letter of referral to another professional. The letter must record treatment offered to date and the issues to be addressed by the other professional. The letter must take account of the stimulus material presented.

The body of the letter must consist of approximately 180-200 words and be set out in an appropriate format. For certain professions, other professional writing tasks of equivalent difficulty may also be set, e.g., responding in writing to a complaint, or providing written information to a specified audience in the form of a letter.



OET

ROLEPLAYER'S CARD NO. 1 — **MEDICINE**

SETTING — Suburban General Practice

PATIENT — You are the parent of a two-week-old child. You have brought him to the doctor today because you are worried about the presence of many small white pimples on his nose and forehead. Your sister's children suffer from severe dermatitis and you fear that your newborn will be similarly affected.

TASK

- Explain your concerns to the doctor.
- Insist that you are very worried that your child's condition is dermatitis and you wish to be referred immediately to a skin specialist for further assessment.
- Be difficult to persuade but not impossibly so. Ask what likelihood there is that the condition will develop into dermatitis.
- Ask for advice on appropriate care for the baby's skin.

OET

CANDIDATE'S CARD NO. 1 — **MEDICINE**

SETTING — Suburban General Practice

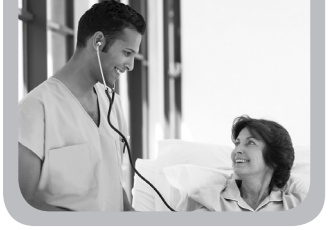
DOCTOR — This parent has brought in his/her two-week-old son, worried about multiple small pimples across the infant's nose and forehead. There is a significant family history (in first-degree relatives) of severe dermatitis. On examination, this young infant shows multiple milia over the nose and forehead, a very common occurrence in newborns.

TASK

- Explain your diagnosis.
- Explain to the parent that the skin condition is common and self-limiting and will resolve spontaneously over the first three to six months of life without needing any special treatment.
- Reassure the parent (e.g., no known association between the occurrence of milia and the likelihood of developing dermatitis in later life) and recommend appropriate skin care.

Speaking sub-test

The production of contextualised professional language is achieved by requiring the candidate to engage with an interlocutor who plays the role of a patient or a patient's carer. The candidate must respond as a professional consultant to two different scenarios played out with the interlocutor. These exchanges are recorded for subsequent assessment. The recording also includes a short 'warm-up' that is part of the Speaking sub-test, though this material is not assessed.



How the test is scored

You will receive a Statement of Results which shows your grade for each of the four sub-tests, from A (highest) to E (lowest). Each of the four sub-tests is assessed in a specific way.

Writing and Speaking

Your performances on the Writing and Speaking sub-tests are each rated by at least two trained Assessors at the OET Centre. Audio files and scripts are assigned to Assessors at random to avoid any conflict of interest. Your test-day Interlocutor is not involved in the assessment process.

Writing and Speaking Assessors are monitored for accuracy and consistency, and the scores they award are adjusted to take into account any leniency or severity. If two Assessors award different scores to your performance, your script and/or audio file will be referred to at least one other senior Assessor not previously involved in your assessment.

For the Writing sub-test, each Assessor scores your performance according to five criteria: Overall Task Fulfilment, Appropriateness of Language, Comprehension of Stimulus, Linguistic Features (Grammar and Cohesion), and Presentation Features (Spelling, Punctuation, and Layout). The five criteria are equally weighted. Grade B for Writing requires a high level of performance on all five criteria.

For the Speaking sub-test, each Assessor scores your performance according to five criteria: Overall Communicative Effectiveness, Intelligibility, Fluency, Appropriateness, and Resources of Grammar and Expression. The five criteria are equally weighted. Grade B for Speaking requires a high level of performance on all five criteria.

Listening and Reading

Your answer booklets for the Listening sub-test and for Reading Part A are marked by trained Assessors at the OET Centre. Answer booklets are assigned at random to avoid any conflict of interest. Your answer sheet for Reading Part B is computer scanned and automatically scored.

Listening and Reading Assessors use a detailed marking guide which sets out which answers receive marks and how the marks are counted. Assessors use this guide to decide for each question whether you have provided enough correct information to be given the mark or marks available. Assessors are monitored for accuracy and consistency, and the data entry of scores is also double-checked for accuracy.

There is no set score-to-grade conversion for the Listening and Reading sub-tests because there are inevitably minor differences in the difficulty level across tests. The grade boundaries for each version of the test are set so that all candidates' results relate to the same scale of achievement. Grade B for Listening and grade B for Reading both require the use of a range of skills, including the ability to understand main ideas, factual information, opinions and attitudes, and to follow the development of ideas.

What is the Speaking sub-test?

OET Speaking sub-test is a test of the candidate's verbal use of English language to communicate in a simulated health-related consultation.

The consultation is between a patient and a health practitioner with the candidate taking the role of the practitioner and the interlocutor taking the role of patient, carer, etc. The candidate and the interlocutor interact face-to-face.

The Speaking sub-test lasts for approximately 20 minutes. It consists of two role-plays.

The role-play tasks are profession specific. In each role-play, the candidate takes the professional role. For example, a nursing candidate takes on the role of nurse in the role-play, and a dentistry candidate takes on the role of dentist. The interlocutor plays a patient/client or sometimes a patient's relative or carer. For veterinary science the interlocutor is the owner or carer of the animal.

In each Speaking sub-test, the interlocutor first checks the candidate's identity and profession. This is followed by a short warm-up conversation about the candidate's professional background. The warm-up conversation will **NOT BE MARKED**. Next the role-plays are introduced. The candidate has 2-3 minutes to prepare for each role-play. Each role-play takes about five minutes.

Information for each role-play is printed on a card, which the candidate keeps during the Speaking sub-test. The candidate may write notes on the card if he/she wants. The role-play information explains the situation and what the candidate is required to do. Any questions about the content of the role-play or how a role-play works may be asked for clarification before the role-play starts.

The interlocutor is trained to ensure the structure of the Speaking sub-test is consistent for each candidate. The interlocutor also uses detailed information on his/her role-play card. The interlocutor **DOES NOT** assess the candidate.

The role-plays are based on typical workplace situations and reflect the demands made on the professional in those situations. A bank of role-plays is used at each administration and each candidate is given two role-plays.

The Speaking sub-test is in three parts:

1. Warm-up conversation (this is not assessed)

- format of the test explained
- candidate helped to relax
- questions asked about areas of professional interest, previous work experience, future plans, etc.

2. First role-play (assessed)

- candidate handed role-play card
- candidate has 2-3 minutes to prepare
- candidate can ask questions to clarify before role-play starts
- role-play is conducted (approximately 5 minutes)

3. Second role-play (assessed)

- The above procedure is repeated using a different role-play

The whole Speaking sub-test is audio recorded and the audio recording is assessed. The assessment is given on the candidate's performance in the two role-plays only (not the warm-up conversation).

OET is a test of English language, not a test of professional knowledge.

Assessment criteria

The candidate's speaking performance is assessed by two qualified assessors who have been trained in OET assessment procedures.

The Speaking sub-test recordings are assessed in Melbourne, Australia. All recordings are double marked.

Important: The interlocutor on the day of the test **DOES NOT** assess the candidate's performance.

Rationale

An important part of a health professional's role is the ability to communicate effectively in speech with his/her patients or clients. The role-plays allow the candidate to take his/her professional role and demonstrate the ability to deal with common workplace situations. These situations may include elements of tension which are a normal part of the real-life context, for example, anxious or angry patients, patients who misunderstand their situation, etc.

The two role-plays, each with a different scenario, provide two separate opportunities for the candidate to demonstrate spoken proficiency, therefore giving a broad view of the candidate's spoken skills.

Role-play tasks are designed to give candidates opportunities to demonstrate their language ability, for example, to:

- negotiate meaning with the interlocutor who is playing the role of the patient (e.g., reassure a worried patient, clarify a medical explanation, manage an upset patient, etc.).
- explain medical conditions/treatments and terminology in an accessible way.
- rephrase ideas and opinions in different ways to try and convince a patient.
- ask and answer questions to and from the patient.
- engage with a variety of patient types (different ages, personalities, different health concerns, etc.).

The candidate's performance in the two role-plays is assessed against the following five criteria:

- Overall Communicative Effectiveness
- Intelligibility
- Fluency
- Appropriateness
- Resources of Grammar and Expression

Each of the five criteria is discussed in further detail next (P. 10-21) and illustrated using sample extracts of 'high' and 'low' candidate performances.

NOTE: The following extracts are examples only. Assessors are carefully trained to assess candidates' sustained performance across both role-plays.

Look at the role-play cards below. Read the tasks shown in Sample role-plays 1 and 2 carefully. Performances of Sample role-plays 1 and 2 are demonstrated on the following pages (11-21). (Example 1 = HIGH performance, Example 2 = LOW performance)

Sample role-play 1

ROLEPLAYER CARD

SETTING Community Health Centre

PATIENT Your doctor has referred you to the community health nurse for advice about lifestyle changes, as your blood pressure is high. You know that you haven't been paying much attention to your health recently but you have a stressful job as a lawyer and your busy schedule makes it difficult for you to exercise on a regular basis. You also smoke a packet of cigarettes a day and enjoy a couple of alcoholic drinks after work; although you know this isn't good for you, it helps you to cope with the stress of work.

TASK
- Answer the nurse's questions about your lifestyle honestly, but be a bit defensive in your replies.
- Be resistant at first to any of the nurse's suggestions about how you can modify your lifestyle, given the stress you are up against at work.
- Finally, agree to some of the changes suggested but continue to be adamant that you cannot give up smoking.

CANDIDATE CARD

SETTING Community Health Centre

NURSE The client has high blood pressure and has been referred to you for advice on lifestyle changes aimed at reducing it. The medical report indicates that the client smokes, drinks and does not take any regular exercise. He/she has a stressful job as a lawyer.

TASK
- Question the client about his/her smoking, drinking and exercise habits.
- Suggest ways in which your client can modify his/her lifestyle to improve his/her health.
- Be sympathetic to your client's reasons for being reluctant to change his/her habits but insist that at least some changes are necessary for the sake of his/her health.

Sample role-play 2

ROLEPLAYER CARD

SETTING Outpatient Clinic at a Public Hospital

PATIENT The patient is your ten-year-old son, Tom, who has broken his arm while playing volleyball at school. You are currently waiting for Tom's arm to be treated; the doctor has already told you that Tom has a greenstick fracture. You are very worried.

TASK
- Explain how the accident happened.
- Ask what is meant by a 'greenstick' fracture.
- Ask about the treatment and how best to care for Tom's broken arm once he arrives home.
- Ask about the healing process. How long will it take for Tom's arm to heal?
- Ask if Tom's growth will be affected by this accident, and if he will be able to play volleyball again.

CANDIDATE CARD

SETTING Outpatient Clinic at a Public Hospital

NURSE The patient is Tom, a ten-year-old boy who broke his arm earlier today when playing volleyball at school. Tom's mother/father is with him and is asking you for information.

TASK
- Ask how the accident happened.
- Explain what is meant by a greenstick fracture (an incomplete break in a bone).
- Explain the procedure involved in the setting of Tom's arm and why, once he arrives home, it would be advisable for him to keep his arm in a sling.
- Explain the healing process.
- Discuss Tom's future growth and whether he will be able to play volleyball again.

1. Overall Communicative Effectiveness

This criterion assesses how well a candidate is able to maintain meaningful interaction during the role-play.

Assessors will use this criterion to evaluate the overall quality of the candidate's performance.

A strong proficiency candidate will:

- show sufficient language skills to effectively complete the task as required by the role-play.
- demonstrate language skills that a health professional would use to guide and advise a patient during a consultation, e.g., informing, explaining, reassuring, advising, enquiring.
- be active in maintaining the conversation, e.g., he/she will initiate the consultation and not simply wait for the 'patient' to ask him/her questions or pass on information.
- interact meaningfully with the interlocutor (as patient).
- communicate confidently in the situation provided by the role-play.
- use appropriate medical terms but explain these terms (e.g., conditions, treatments, etc.) in ways that a normal patient can understand.
- demonstrate that he/she can take the information from the role-play card and information the 'patient' provides and use it effectively to complete all aspects of the task.
- complete the task in the allocated time. Approximately 5 minutes is allowed for each role-play.
- successfully complete the task by using the prompts provided on the role-play cards.

Now, look at Example 1 which demonstrates HIGH performance. Some key points are described in each example in relation to the criterion: Overall Communicative Effectiveness.

Example 1

HIGHER
LOWER

"

CANDIDATE: Do you have any time to eat breakfast?

PATIENT: Er... sometimes.

CANDIDATE: Do you drink coffee or smoke instead of breakfast?

PATIENT: Yes, that's right.

CANDIDATE: Well, considering your blood pressure being higher, you have two options. The first option is, you're going to have medication, which would be the last solution. The second option, the better option I think, is changing your lifestyle. You do not need to change everything in your life, but you need to make it better...

PATIENT: Yeah, ok.

CANDIDATE: What do you think it's easier or better for you? Where do you want to start? Do you want to start with... your eating habit?

PATIENT: Alright.

CANDIDATE: And, do you think you can give up smoking?

PATIENT: Oh, I just can't. I really love smoking, so...

CANDIDATE: What about reducing smoking? Not twenty cigarettes, but fifteen?

PATIENT: Yeah, I just can't give it up altogether. But I could try to cut down, I guess.

"

The candidate is active in initiating the conversation.

There are some minor errors, however, the level of intrusiveness is very low.

Advice is given in an accessible manner.

Language skills are appropriate for the tasks within the role-play.

Interaction is meaningfully carried out.

Conversation is confidently maintained.

Now, look at Example 2 which demonstrates LOW performance. Some key points are described in each example in relation to the criterion: Overall Communicative Effectiveness.

Example 2

HIGHER
LOWER

Communication is significantly restricted by frequent errors.

Consultation is restricted in range and flexibility by many lapses.

The candidate manages to answer the patient's questions, but not in a clear way.

Some components of task are left unfulfilled.

The candidate manages to maintain the interaction.

Linguistic resources are restricted.

"

CANDIDATE: Umm... it's very hard to for school kids... to... er... er... stuck in the home... and nothing to do. So just... er... need rest and not using arm very much and use a sling. And no operation right now. And... also have... he can take painkiller.

PATIENT: Er... how much painkiller should I give him?

CANDIDATE: Er... I'm not too sure now because... doctor will... er... prescript... for that drug... medicine. So I'll check that for you later.

PATIENT: Ok. Alright. Er... so... ok... so it should take about a month for him to heal.

CANDIDATE: At least... er... about a month, and... yeah... doctor will check him... in one month... and... and also take... X-ray. And he... the doctor will decide... er... and tell you how long... er... need rest more... and... when he can play volleyball... again.

PATIENT: Ok.

CANDIDATE: ... because maybe he... is... enjoying... to playing volleyball...?

PATIENT: Ah, he does. Er... In fact, I'm wondering er... will he be able to play volleyball again?

CANDIDATE: Sorry?

PATIENT: Er... will he be able to play... volleyball?

CANDIDATE: Usually... er... most people recovery around... in three months to six months depend on patient, and depend on the which place... they got injury, because if... er... a lots of using place... er... maybe stay less too longer. But... depend on patient. I'm sorry I can't tell exactly.

"

2. Intelligibility

This criterion assesses how well a candidate's speech can be heard and understood. It concerns the impact of such features of speech as pronunciation, rhythm, stress, intonation, pitch and accent on the listener.

Assessors will use this criterion to evaluate the candidate's production of comprehensible speech.

A strong proficiency candidate will:

- use natural flow of speech, giving stress to particular words within sentences to emphasise meaning, e.g., 'I'm unable to do THOSE tests in THIS clinic'.
- use natural flow of speech, giving correct stress to syllables within words so that they are identifiable to the listener, e.g., 'I will reCORD your results'. 'This is an accurate REcord of your results'.
- show control of intonation (voice falling or rising) and stress (appropriate force, length, emphasis or loudness) to enhance meaning and strengthen the communication he/she is wanting to provide.
- pronounce words clearly, for example:
    1. consonants at the end of words or syllables (e.g., 'head', 'weakness').
    2. consonants that distinguish different meanings of similar words (e.g., 'worry', 'worries', 'worried').
    3. consonant sounds at the beginning of words (e.g., /v/ as in '**v**omit', /b/ '**b**ill' versus /p/ '**p**ill').
    4. syllables within words (e.g., 'dang(er)ous', 'a coup(le) of days').
    5. clear initial consonant blends 'problem', 'bleeding'.
    6. word stress in longer words (e.g., 'PAINkiller' not 'painKILLer', 'HOSpital' not 'hosPItal').
    7. vowel sounds (/əʊ/ 'n**o**te' versus /ɒ/ 'n**o**t').
- minimise any intrusive sounds, rhythm and accent which may be influenced by his/her mother tongue.
- show the ability to link words together naturally. For example, there are often no 'spaces' between words in phrases like, 'in_about_an_hour'.

Now, look at the following examples. Examples 1 and 2 demonstrate HIGH and LOW performances respectively. Some key points are highlighted in each example in relation to the criterion: Intelligibility.

Example 1

HIGHER
LOWER

"... I **think** you can **find** a few friends who **regularly** go for a **walk;** ↘ you can **start** with them. ↘ And if you **reduce** smoking and cut the amount of **coffee** you drink a **day**, it would **help** your **blood** pressure level. ↘ **Start** to drink more **water** and do some **exercise**, your **blood** pressure will be **better** in a month. ↘"

Comment

Prosodic features (stress, intonation ↘ and rhythm) are used efficiently. The speech is easily understood even though the evidence of the first language is present. Certain words are linked together naturally.

Example 2

HIGHER
LOWER

"

Wrong		Correct
in**jury**	=>	**in**jury
severe	=>	se**vere**
in**flu**ence	=>	**in**fluence

"

Comment

Issues with non-standard word level stress and incorrectly pronounced vowels interfere with the listener's ability to understand all information. This affects 'Intelligibility'.

"
- ... er... she injured her spine (pronounced as 'sp**ʌ**n')... is a very important... organ...

[sp/**aɪ**/n]

- ... may be several months, she can't mobilise (pronounced as 'mob**ju**:laiz') herself...

[moub/**ə** /laiz]

"

Comment

Vowels are not pronounced correctly, which confuses the patient. The vowel sound in 'spine' [sp/aɪ/n] is not the same as the vowel in 'spun' [sp/ʌ/n], but should be pronounced as [sp/aɪ/n]. The vowel sound in 'mobilise' [moubəlaiz] is not the same as the vowel in 'bureaucrat' [bju:rəkræt], but should be pronounced as [moubəlaiz].

3. Fluency

This criterion assesses how well a candidate's speech is delivered in terms of rate and flow of speech.

Assessors will use this criterion to evaluate the degree to which a candidate is able to speak continuously, evenly and smoothly – without excessive hesitation, repetition, self-correction or use of 'fillers'.

A strong proficiency candidate will:

- maintain a natural speed to make it easier for the listener to follow the message (not too slow, not too fast).
- use even speech (not broken up into fragments) and limit hesitations or speaking in 'bursts' of language.
- avoid overusing sounds (e.g., 'err', 'um', 'ah') and words (e.g., 'OK', 'yes') to fill in gaps.
- use a smoother flow of speech, stressing syllables appropriately and linking words/syllables together.
- use pauses appropriately, for example:
    1. to make his/her meaning clear, e.g., for emphasis.
    2. to separate clearly the points he/she is making.
    3. to think about what he/she is going to say next.
- avoid restarting sentences or repeating words and phrases as he/she corrects himself/herself.

Look at the following examples. Examples 1 and 2 demonstrate HIGH and LOW performances respectively. Some key points are described on each example in relation to the criterion: Fluency.

Example 1

HIGHER
LOWER

"... I think you can find a few friends who regularly go for a walk; you can start with them...(omission)... . Start to drink more water and do some exercise, your blood pressure will be better in a month."

Comment

The flow of the speech is good, not too fast or not too slow.

The speech is even and hesitation is rarely evident.

There is little use of 'fillers' (e.g., 'err', 'um', 'OK', etc.).

Restarting sentences is rare.

Example 2

HIGHER
LOWER

"
- That is a common concerned from some patients...because they don't know any...don't know more... don't know many medications... something like that...
- You can also give her some... ~~give~~ **let** her inhaler some steams...she can inhaler the steam... That can make her ~~to breath~~ **breathe** easily...

"

Comment

There is some hesitation that affects fluency.

This candidate often pauses during his/her speech while he/she prepares what to say next.

This 'breaking up' of the message can affect the listener trying to decode it. This affects 'Fluency'.

How to improve

Try to work on a smoother delivery without so many false starts and reformations.

4. Appropriateness

This criterion assesses how well a candidate uses language, register and tone that are appropriate to the situation and the patient.

Assessors will use this criterion to evaluate the degree to which the individual words, grammar and style of speech the candidate selects are appropriate to the particular situation and context.

A strong proficiency candidate will:

- use suitable, professional language.
- use appropriate paraphrasing and re-wording if necessary to explain, in simple terms, technical procedures or medical conditions to a patient who may have little knowledge of these.
- adapt their style and tone to suit the particular situation of the role-play, e.g., giving bad news versus giving positive news or using language suitable for talking to an older person versus a younger person.
- respond appropriately to what the 'patient' says during the role-plays, e.g., the candidate's responses are logically linked with the patient's questions or concerns.
- use language that might reflect the professionalism a health practitioner might require when dealing with patients, e.g., not overly-familiar or informal.
- demonstrate that he/she has the language skills to deal well with complicated situations, e.g., complaints, difficult patients, patients who need convincing, etc.
- use appropriate phrases that are suited to common functions found in medical exchanges, e.g., to 'reassure', 'encourage', 'be supportive', 'explain', etc.
- show awareness of the patient's sensitivities to the condition or information the candidate gives.

Now, look at the following examples. Examples 1 and 2 demonstrate HIGH and LOW performances respectively. Some key points are described on each example in relation to the criterion: Appropriateness.

Example 1

HIGHER
LOWER

"... What do you think is easier or better for you? Where do you want to start? Do you want to start with ... your eating habit?"

"... and you do not need to do some intensive fitness activities. I think it's enough if you start with walking for half an hour everyday."

Comment

This candidate uses a good strategy to convince the unwilling patient (e.g., using questions rather than imperative forms to encourage the patient).

An appropriate tone is used to encourage the patient.

Example 2

HIGHER
LOWER

"
- If...she doesn't get treatment ~~effectively~~... it may ~~be worsen~~... (effective; get worse)
- As far as we know, the antibiotic ~~doesn't~~ really helpful for viral infections... (is not)
"

Comment

The misuse of natural phrases and expressions is affecting 'Appropriateness'. The underlined phrase indicates considerable doubt, whereas antibiotics definitely do not work for viral infections.

"
- If you don't keep eye on this disease...you might ~~get~~ blind unfortunately. But if you keep ~~to do~~ your blood sugar level and to keep eye on diet... (an; go; checking; your; an)
"

Comment

At times the message is interrupted by word choice errors. This affects 'Appropriateness'.

How to improve

Take care with phrases that can be easily confused. Meaning breaks down if the phrase is only partially correct.

5. Resources of Grammar and Expression

This criterion assesses the level and extent of the candidate's grammar and vocabulary resources and their appropriate use.

Assessors will use this criterion to evaluate the range and accuracy of the language resources the candidate has applied in the performance to convey clear meaning.

A strong proficiency candidate will:

- use appropriate structures to make what he/she is saying coherent, for example, outlining options or choices to a patient (e.g., 'There are several options you can consider. Firstly, in the short term, …').
- show flexibility by using different phrases to communicate the same idea, if necessary, to make it clearer.
- form questions correctly, particularly those questions that are often used in health professional/patient dialogues (e.g., 'How long have you been experiencing this?', 'When did the symptoms start?').
- minimise grammatical inaccuracy to enhance communicative effectiveness.
- use more complex structures and expressions confidently (e.g., idiomatic speech, sentences with multiple clauses, etc.), i.e., not just a series of simple utterances.
- use a wide variety of grammatical structures and vocabulary that reflects the depth and range of their linguistic resources.
- show accurate control of grammatical features including, for example:
    1. correct word order (e.g., 'She broke her tooth' not 'She tooth her broke').
    2. correct use of pronouns/relative pronouns (e.g., 'Tell her it's ok if she (not he) waits then comes back to see me when she (not he) feels better').
    3. correct word choice (e.g., 'Your daughter is breathing more rapidly/repeatedly/regularly' (all have different meanings)).
    4. not omitting words that could affect clear meaning (e.g., 'I recommend that you consider several options including crown, fillings and inlays' not 'I recommend about crown, filling, inlay').
    5. correct use of prepositions (e.g., 'I can explain to you about asthma' not 'I can explain you about asthma').
    6. correct use of articles (e.g., 'A form is completed and then given to the Pharmacist' not 'Form is completed and then given to Pharmacist').
    7. use correct word form (e.g., 'Smoking is dangerous for your health' not 'Smoking is danger for your health').
    8. correct use of countable and uncountable expressions (e.g., 'not many side effects' not 'not much side effects').
    9. use appropriate structures to convey information about time and the sequence of past or future events (e.g., 'We have X-rayed your arm and the results will be available today/next week' not 'We X-ray your arm and the results available').

Now, look at the following examples. Examples 1 and 2 demonstrate HIGH and LOW performances respectively. Some key points are described on each example in relation to the criterion: Resources of Grammar and Expression.

Example 1

HIGHER
LOWER

"... You have two options. The first option is, you're going to have medication, which would be the last solution. The second option, the better option I think, is changing your lifestyle. You do not need to change everything in your life, but you need to make it better..."

Comment

The available options for the patient are outlined in a coherent manner (e.g., 'You have two options. First...').

The number of errors are not intrusive.

Information is given in a confident manner.

Different structures are used to communicate the same idea effectively (e.g., '...is changing your lifestyle. You do not need to change everything...').

Example 2

HIGHER
LOWER

"
- **No, I'm not forcing, this is option...** (you)
- **If you have some pain, try not to use too much because I will put some dressing...'** (an, it, on it)

"

Comment

Many sentences are incomplete. Watch out for pronouns such as 'you', 'it' and prepositions such as 'put something on (something)'.

"**You need to be free of infections. What you can do is to take some cleaning gloves every time and do something with clean clothes and something like that...**"

Comment

Many simple words are used repetitively, affecting "Resources of Grammar and Expression'. In the above example, 'something' is overused, indicating gaps in vocabulary.

How to improve

Be more specific with word choice.

Information about role-play cards

Candidates will have an opportunity to read through the role-play card before starting each role-play. Both role-play cards are laid out in a similar way. At the top of the role-play card is information about the setting (i.e. where the conversation is taking place). Candidates receive information on each role-play card, which he/she keeps while doing the role-play. Candidates may write notes on the role-play card if they want to.

The role-play card explains the situation and what candidates are required to do. If candidates have any questions about the content of the role-play or how a role-play works, they may ask for clarification before starting.

The top paragraph contains background information about the patient and his/her situation. It will be made clear if the interlocutor is taking on the role of the patient or somebody talking on behalf of the patient (i.e., the patient's carer, parent, etc.). The bottom half of the role-play card contains information to assist candidates in what they need to mention during the role-play. Each role-play card contains approximately 100-150 words (prompts/notes to guide candidates during the role-play).

Candidates should use the prompts/notes on the role-play card to guide them through the role-play:

- What is your role?
- What role is your interlocutor playing – patient, parent/son/daughter, carer?
- Where is the conversation taking place?
- What is the current situation?
- How urgent is the situation?
- What background information are you given about the patient and the situation?
- What are you required to do?
- What is the main purpose of the conversation (e.g., explain, find out, reassure, persuade)?
- What other elements of the situation do you know about (e.g., the patient appears nervous or angry, you don't have much time)?
- What information do you need to give the patient (remember, though, this is not a test of your professional skills)?

Using the practice tests

- Copy the role-play.
- Ask a friend or colleague to play the role of the patient (or patient's carer, etc.).
- Take the role of the health professional.
- Ask another friend or colleague to observe the role-play and give you feedback on your performance.
- Read the information on the role-play card carefully.
- You have to deal with the case details as outlined on the role-play card by asking and answering questions put to you by the patient or client.
- Speak as naturally as possible.
- Remember it is important to be interested in the welfare of the patient and to reassure the patient or relation of the patient that the treatment being proposed is appropriate.
- Keep to the time limit of 5 minutes (approximate) for each role-play.
- Ask the friend or colleague who observed for comments and feedback.

Useful language

Greeting Introduction

- Good morning/afternoon/evening.
- Nice to see you (again).
- How are you today?
- My name is Dr .../I'm Dr ...
- Thanks for coming to see me today.
- Pleased to meet you (response to patient's introduction).

Getting information

Starting the interview:

- What brings you along here today?
- What brought you here today?
- What seems to be the trouble/problem?
- How can I help you?
- What can I do for you?
- What seems to be bothering you?

Asking about location of the problem:

- Where is the sensation?
- Can you tell me where it hurts?
- Where do you feel sore?
- Where does it feel sore?
- Which part of the/your body is affected?
- Show me where the pain is.
- Tell me where the pain is.

Asking about duration:

- When did it start?
- How long have you had it?
- How long have you been feeling like this?
- How often has this been occurring?
- How long have you been suffering from this problem?
- When did the problem start?

Asking about severity of pain or type of pain:

- Is the pain dull or sharp?
- What is the pain like?
- Could you describe the pain?
- How severe is the pain?
- Does it disturb you at night?
- Does it feel numb?
- Does it occur all of the time or just now and again?

Questioning

To clarify/to get details:

- Have you had any...?
- Does the discomfort appear to be brought on by anything in particular?
- What do you do when you get the pain?
- Do you ever get pain at night?
- Does anything special make it worse?
- Does anything seem to bring it on/aggravate the problem?
- Is there anything that seems to relieve this?

Prescribing

Tests, medicine, treatment:

- I think we would start with...
- I will give you a prescription for...
- I will give you a referral for...
- I'll write a referral letter to...
- I'm going to ask you to fill a prescription for...
- We'll run some tests to see...

Reassurance

- I can understand your concerns, but...
- I'm sure you won't have any more trouble...
- Don't worry, it'll go away by itself/in a few days/with some rest...
- Rest assured, this is quite common...
- There is nothing to be overly concerned about.

Advising Suggesting

- What I think we'll do is ...
- What I suggest you do is ...
- It is worthwhile...
- I advise you...
- We could make a time to follow up on that.
- It's a good idea to ...

Leave-taking

- Pleasure to meet you.
- Nice to meet you, ...
- Let's leave it there.
- All the best, ...
- We'll see you next time/soon.
- Thanks very much for coming to see me.

Check understanding

- Do you have any questions?
- Have you ever heard of ...?

Feedback

Respond to patient's questions:

- Were there any other questions?
- Does this sound ok/like an acceptable plan?

OCCUPATIONAL ENGLISH TEST

Speaking sub-test

Practice test 1

OET Sample role-play

ROLEPLAYER CARD NO. 1 **NURSING**

SETTING

Hospital Ward

CARER

You are the parent of a 12-year-old girl. Your daughter had a convulsive seizure (epileptic fit) earlier in the day at school and was brought to the hospital by ambulance. A nurse has just come to tell you that your daughter is about to be discharged from the hospital and you decide to ask him/her for information about what to do were your daughter to have another convulsive seizure (epileptic fit).

TASK

- Ask what epilepsy is and what causes epilepsy.
- Explain your concern that your daughter may have other convulsive seizures, and that you need to know what you should, or should not, do.
- Ask what you should do if your daughter appears disorientated or begins behaving strangely.
- Ask when you should call an ambulance.
- Ask what other support might be available.

© Cambridge Boxhill Language Assessment

OET Sample role-play

CANDIDATE CARD NO. 1 **NURSING**

SETTING

Hospital Ward

NURSE

The parent of a 12-year-old girl asks for your co-operation in answering questions about a convulsive seizure (epilepsy) and what to do if his/her daughter had another seizure following her discharge from hospital sometime later today. Listen carefully to the parent's questions. Do your best to answer the questions. Stress the need to stay calm, and not become over-anxious.

TASK

- Briefly explain epilepsy (disruption of electrochemical brain activity) and general symptoms (e.g., muscle jerking, sometimes post-seizure tiredness or incontinence, etc.).
- Explain what the parent should do if the daughter has another convulsive seizure at home (e.g., lie on side, remove harmful objects, no objects near mouth, check breathing, etc.).
- Explain when to call an ambulance.
- Explain that there is an Epilepsy Foundation which could, on request, give the parent more information about epilepsy. Its details will be in the local telephone book or on the internet.

© Cambridge Boxhill Language Assessment

OET Sample role-play

ROLEPLAYER CARD NO. 2 NURSING

SETTING Home Visit

PATIENT You are a 90-year-old client, who has lived alone fairly independently for many years in a unit on your daughter's property. Your daughter's family are going overseas for two weeks and you are going to a lovely retirement home. You are having an assessment by a community nurse.

TASK

- State that you are happy to go into care so your family can have a pleasant and worry-free trip.
- Emphasise your independence but you are worried about lack of privacy/rigid time tables.
- Agree to give an overview of your daily routine – sleep in, late breakfast, shower whilst daughter in your unit (minimal assistance). Relatives take you out often and you attend a senior citizen's club.
- State that you only take occasional anti-inflammatory or analgesic medication for 'old age' aches/pains.
- Ask the nurse to tell your daughter you are content with the arrangements so they can have the break they deserve.

© Cambridge Boxhill Language Assessment

OET Sample role-play

CANDIDATE CARD NO. 2 NURSING

SETTING Home Visit

NURSE You are visiting a 90-year-old client, who has lived alone fairly independently for many years in a separate unit on a family property. The family are going overseas for two weeks and your client is going to a retirement home to avoid being isolated on a large property. You are conducting an aged care assessment.

TASK

- Compliment the elderly client on the positive and unselfish attitude.
- Reassure the client of privacy/flexible house rules.
- Explain the home's philosophy – making clients feel at home, respecting wishes, unrestricted visiting hours to meet quality of life desires, etc.
- Ask about his/her daily routine (e.g., sleep habits, meals, etc.).
- Highlight the fact that you are noting his/her usual daily routine in your care plan.
- Ask about his/her medication regime.
- Summarise your discussion and assure the client that you will pass on his/her contentment with the arrangements to his/her daughter.

© Cambridge Boxhill Language Assessment

OCCUPATIONAL ENGLISH TEST

Speaking sub-test

Practice test 2

OET Sample role-play

ROLEPLAYER CARD NO. 1 — NURSING

SETTING: Hospital

PATIENT

You are 55 years old. You have End-stage Renal Disease (ESRD) and your health is steadily deteriorating. You are an inpatient in a Renal Ward and feeling lonely and depressed. You are reluctant to talk about your situation as you are scared. You require dialysis three times a week and are on a kidney transplant waiting list.

TASK

- With reluctance, slowly explain your fears about your condition.
- Tell the nurse you hate having dialysis and feel that a donor will never be found in time.
- Demand to be nominated as Not for Resuscitation (NFR) if you have a cardiac arrest.
- Reluctantly agree to see a counsellor.
- Tell the nurse that you want to be able to die with dignity with your family present.

© Cambridge Boxhill Language Assessment

OET Sample role-play

CANDIDATE CARD NO. 1 — NURSING

SETTING: Hospital

NURSE

Your patient is 55 years old. He/she has End-stage Renal Disease (ESRD). You see his/her health is steadily deteriorating. The patient is an inpatient on your Renal Ward and you notice he/she appears lonely and depressed. He/she requires dialysis three times per week and is a candidate for a renal transplant.

TASK

- Ask the patient to tell you how he/she is feeling.
- Encourage the patient to talk about his/her fears about the condition.
- Inform the patient that without dialysis he/she would suffer more.
- Reassure the patient that there is always hope a donor will be found in time.
- Suggest that a counsellor be consulted to support the patient's decision to be listed as Not for Resuscitation (NFR).
- Provide reassurance that, if he/she wants to put his/her request in writing, it is hospital policy for staff to respect that, and having the family present is important.

© Cambridge Boxhill Language Assessment

OET Sample role-play

ROLEPLAYER CARD NO. 2 — NURSING

SETTING: Home Visit

PATIENT

You are a busy 50-year-old waitress, recently diagnosed with Type 2 diabetes. You had a wedge resection 12 days ago and have post-operative infection in your right large toe. You are resting at home and the district nurse is visiting to change your dressing.

TASK

- Admit that you should have followed the surgeon's advice, but did not, so now you have an infection.
- Argue that you need a quick fix to the toe because you need to get back to work; the antibiotics obviously aren't working.
- Agree to use the nurse's list of suggestions and accept that you did not cause your own infection.
- Tell the nurse that you know the risks that diabetics experience with circulation, but would like a brochure on how to manage your illness.

© Cambridge Boxhill Language Assessment

OET Sample role-play

CANDIDATE CARD NO. 2 — NURSING

SETTING: Home Visit

NURSE

Your patient is a busy 50-year-old waitress, recently diagnosed with Type 2 diabetes. The patient had a wedge resection 12 days ago and has post-operative staphylococcus aureus (staph infection, non-MRSA) on the right large toe. You are visiting the home to change the dressing whilst the patient is resting from work duties.

TASK

- Find out about the patient's general daily habits after the operation. Did the patient rest adequately?
- Highlight the fact that sometimes people get post operative infection but it is not really their fault.
- Persuade your patient to rest and that the toe will heal with attendance to dressings, fresh air and daily exposure to the sun, antibiotics, etc.
- Explain the risks of circulation problems for diabetics and sometimes greater risk of infection.
- Supply a patient information brochure to the patient and agree to call again next week.

© Cambridge Boxhill Language Assessment

OCCUPATIONAL ENGLISH TEST

Speaking sub-test

Practice test 3

OET Sample role-play

ROLEPLAYER CARD NO. 1 — NURSING

SETTING: Private Clinic

PATIENT

You are a 52-year-old working long hours in customer service. You are tired and have been experiencing aching feet. Sometimes you have numbness and tingling, which is worse at night. You are worried because your uncle has Multiple Sclerosis and your aunt has Parkinson's disease.

TASK

- Tell the nurse that you are very tired, you suffer from symptoms in the feet, your memory is not too good and when you are stressed your hands shake.
- Explain that your work is stressful, moves at a frantic pace and you have dozens of things to remember, but don't write them down. What can be done?
- Accept that perhaps stress is causing your symptoms.
- Agree to a referral to a podiatrist and seek advice from a doctor.
- Reluctantly express the 'real' reason you are here – you are extremely 'paranoid' about the chance that you may have a neurological disease because of your family history.

© Cambridge Boxhill Language Assessment

OET Sample role-play

CANDIDATE CARD NO. 1 — NURSING

SETTING: Private Clinic

NURSE

You are attending to a 52-year-old who works long hours in customer service. The patient complains of being tired and has been experiencing aching feet. Sometimes he/she has numbness and tingling, which is worse at night. The patient admits to being 'paranoid' as his/her uncle has Multiple Sclerosis and his/her aunt has Parkinson's disease.

TASK

- Explain that working long hours and stress may produce symptoms such as tiredness and poor memory.
- Suggest initial steps to improve sore feet issues (e.g. comfortable shoes, orthopedic shoes; compression stockings, etc.).
- Emphasise that you think the patient may be putting too much pressure on him/herself.
- Explain that you will examine his/her feet for obvious signs of injury and suggest a visit to a podiatrist.
- Advise the patient to arrange a medical appointment.
- Highlight the importance of not self-diagnosing and thinking the worst.

© Cambridge Boxhill Language Assessment

OET Sample role-play

ROLEPLAYER CARD NO. 2 **NURSING**

SETTING General Practice

PATIENT You are a 25-year-old apprentice, who has a pilonidal sinus (cyst) for excision next week. You have quite a bit of pain but you are embarrassed. You have seen the doctor and you are discussing your treatment and preoperative documents with the nurse.

TASK

- Tell the nurse you feel as though you are weak because of this ailment, but need painkillers more often than you take them.
- Emphasise how embarrassed you are, therefore did not pay attention to the doctor when he/she explained what a pilonidal sinus is. Can you still find out?
- Argue that you are very clean and due to your work you shower in the morning and have a bath at night.
- Ask what you need to do after the procedure.
- Reassure the nurse that you will follow the doctor/nursing staff's advice when in hospital, and adhere to the medication regime, rather than suffer pain.

© Cambridge Boxhill Language Assessment

OET Sample role-play

CANDIDATE CARD NO. 2 **NURSING**

SETTING General Practice

NURSE You are preparing a 25-year-old apprentice for excision of a pilonidal sinus (cyst) next week. The patient is in pain but is embarrassed about the problem and thinks taking medication to alleviate pain is a sign of weakness. You are discussing treatment and preoperative documents, following the doctor's appointment.

TASK

- Express concern that the patient has pain and views taking analgesic as a weakness.
- Assure the patient that you understand the embarrassment but doctors and nurses are used to a huge range of medical issues.
- Explain that a pilonidal sinus is a small opening, a tract on the surface of the skin predominantly caused by an ingrown hair, exacerbated by sweat and prolonged sitting.
- Reassure the patient that you were not indicating that he/she is dirty and salt baths post-operatively will help facilitate healing.
- Provide information on post-operative care (e.g., bleeding, signs of infection, importance of taking analgesia appropriately, use of antiseptic dressings, etc.).

© Cambridge Boxhill Language Assessment

Further practice

OET Preparation Portal

The Preparation Portal contains many highly useful resources for OET candidates including:

- Where to buy more OET practice books;
- Attending a free OET Masterclass, containing helpful tips and information for the exam;
- Free sample materials for your profession;
- Top tips for each skill;
- Where to find a preparation course.

The portal can be accessed at: https://www.occupationalenglishtest.org/test-information/preparation-portal/

External websites

There are resources for English-language learners on the web which can help candidates develop the general speaking skills involved in a medical context. Check the language style, appropriate use of the language and some medical terms used in the following websites.

General health:

ABC Australia Health

http://www.abc.net.au/health/ Health Matters – index with links to programmes and features
http://www.abc.net.au/rn/allinthemind/ All in the Mind
http://www.abc.net.au/rn/healthreport/ Health Report
http://www.abc.net.au/rn/lifematters/ Life Matters
http://www.abc.net.au/health/minutes/ Health Minutes

BBC World Service Health

http://www.bbc.co.uk/worldservice/programmes/health_check.shtml Health Check
http://www.bbc.co.uk/worldservice/programmes/science_in_action.shtml Science in Action

Newsletters

You could subscribe to the regular health-related newsletters:
http://www.abc.net.au/health/subscribe/default.htm

Englishmed.com

http://www.englishmed.com/ English learning resources with a medical focus

Cambridge Boxhill Language Assessment is not responsible for the content of external websites.

The OET Centre
PO Box 16136
Collins St West
VIC 8007 Australia
Tel: +61 3 8656 4000
www.occupationalenglishtest.org

CAMBRIDGE ENGLISH
Language Assessment
Part of the University of Cambridge

BOX HILL INSTITUTE
Global Educators™

The Occupational English Test (OET) is designed to meet the specific English language needs of the healthcare sector. It assesses the language proficiency of healthcare professionals who wish to register and practise in an English-speaking environment.

OET is owned by Cambridge Boxhill Language Assessment Trust (CBLA), a venture between Cambridge English and Box Hill Institute. Cambridge English Language Assessment is a not-for-profit department of the University of Cambridge with over 100 years of experience in assessing the English language. Box Hill Institute is a leading Australian vocational and higher education provider, active both in Australia and overseas.